

PERFORMANCE WORK STATEMENT (PWS)
Arkansas Army National Guard (ARARNG)
Medical and Dental Readiness Services
04 May 2026

1.0 General:

1.1 Scope: The contractor shall provide all personnel, equipment, tools, materials, supervision, and quality control necessary, except as specified as Government Furnished, to perform Medical and Dental Services, as defined in this PWS. The Arkansas Army National Guard (ARARNG) expects the Contractor to provide excellent support by providing the right expertise at the right place at the right time.

Mission: The Office of the State Surgeon has the primary responsibility to develop, manage, and execute all medical readiness operations to enhance ARARNG Initiatives. Our Role: To maintain individually ready, properly trained and equipped units, available for prompt mobilization of war, national or domestic emergency, or as otherwise required by state law.

1.2 Background: Individual Medical Readiness (IMR) and Medical Readiness Category (MRC) are the basic principles of force-sustainment for ARARNG forces. Medical & dental health is vital for force health protection and failure to achieve a standard level of medical and/or oral health could preclude a Soldier from deploying. Medical & Dental Readiness is a command responsibility.

Department of Defense established a uniform dental classification in HA Policy 02-11. National Guard Bureau (NGB) issued guidance to provide dental treatment to Soldiers outside of alert phase of the Soldier Readiness Model (SRM) by issuing NGB-ARS detailing the Army Selected Reserve (SELRES) Dental Readiness System (ASDRS) policy. DoD policy requires dental screening examinations and remediation to be accomplished and documented on 90%+ of service members. Members of the ARARNG shall receive an annual dental screening examination to determine their dental classification. Soldiers are eligible for Dental Treatment if they are Class 3 Status from examination. Soldiers outside of alert status are also eligible for treatment if they are a Class 3 Status from examination. It is essential to provide Dental Treatment that may be required at the MEDICAL/DENTAL READINESS & SRP site to ensure the highest Dental Deployable Rate for the Non- Mobilizing & Mobilizing Soldiers.

Department of The Army established a requirement for a routine, annual Periodic Health Assessment (PHA) to be conducted for all members of the Active Duty and the Selected Reserves. This program focuses on optimizing the health and wellness of Soldiers.

1.3 Period of Performance (PoP): The Period of Performance shall be two days, 30-31 May 2026.

1.4 General Information:

1.4.1 Place and Performance of Services: The contractor shall be prepared to provide services at the locations listed (Technical Exhibit 3). Events will be scheduled Monday through Friday and or on Saturday and Sunday, except on recognized US holidays or when the Government facility/installation is closed due to local or national emergencies, administrative closings, or similar

Government-directed facility/installation closings. Performance shall be at predetermined locations throughout the state of Arkansas (Technical Exhibit 3). Contractor shall be able to respond within a minimum of 3 days' notice anywhere within the state of Arkansas. Event operation hours shall be in three-hour blocks up to four consecutive blocks not to exceed four consecutive blocks per day unless special arrangements are made. Physical addresses will be provided to contractor upon determination of event need and will be provided in each delivery order. The contractor shall always maintain an adequate work force for the uninterrupted performance of all tasks defined within this PWS when the Government facility/installation is not closed for the above reasons.

1.4.1.1 Telework: Not authorized.

1.4.1.2 Unscheduled gate closures by the Security Police may occur at any time causing all personnel entering or exiting a closed installation to experience a delay. This cannot be predicted or prevented. Contractors are not compensated for unexpected closures or delays. Vehicles operated by contractor personnel are subject to search pursuant to applicable regulations. Any moving violation of any applicable motor vehicle regulation may result in the termination of the contractor employee's installation driving privileges.

1.4.1.3 The contractor's employees shall become familiar with and obey the regulations of the installation, including fire, traffic, safety, and security regulations while on the installation. Contractor employees should only enter restricted areas when required to do so and only upon prior approval. All contractor employees shall carry proper identification with them at all times and shall be subject to such checks as may be deemed necessary. The contractor shall ensure compliance with all regulations and orders of the installation, which may affect performance. The Government reserves the right to direct the removal of an employee from Government property or revoke access to Government systems for misconduct, security reasons, or any overt evidence of communicable disease. Removal of contractor employees for reasons stated above does not relieve the Contractor from responsibility for total performance of this contract.

1.4.2 Recognized Holidays: The following are recognized United States (US) holidays. The contractor shall not perform services on these days:

1.4.2.1 New Year's Day: January 1st

1.4.2.2 Martin Luther King, Jr.'s Birthday

1.4.2.3 President's Day

1.4.2.4 Memorial Day

1.4.2.5 Juneteenth National Independence Day: June 19th

1.4.2.6 Independence Day: July 4th

1.4.2.7 Labor Day

1.4.2.8 Columbus Day

1.4.2.9 Veteran's Day: November 11th

1.4.2.10 Thanksgiving Day

1.4.2.11 Christmas Day: December 25th

1.4.3 Quality Control (QC): The contractor shall develop and maintain an effective QC Plan (QCP) to ensure services are performed in accordance with this PWS. The contractor shall develop and implement procedures to identify, prevent, and ensure non-recurrence of defective services. The

contractor's QCP is the means by which it assures itself that its work complies with the requirements of the contract. As a minimum, the contractor shall develop QC procedures that address the areas identified in Technical Exhibit 1, Performance Requirements Summary (PRS). A final QCP shall be submitted to the Contracting Officer Representative (COR) NLT 10 days after contract award to be reviewed and approved by the ARARNG. After acceptance of the QCP, the contractor shall obtain the Contracting Officer's (KO's) acceptance in writing of any proposed changes to its QCP.

1.4.4 Quality Assurance (QA): The Government will evaluate the contractor's performance under this contract in accordance with the Quality Assurance Surveillance Plan (QASP). This plan is primarily focused on what the Government will do to ensure that the contractor has performed in accordance with the performance standards. It defines how the performance standards will be applied, the frequency of surveillance, and acceptable quality level(s) or defect rate(s).

1.4.5 Installation Access and Security Requirements. The contractor shall comply with all applicable installation/facility access and local security policies and procedures, which may be obtained from the COR. The contractor and all associated subcontractor employees shall provide all information required for background checks to meet installation access requirements to be accomplished by installation Provost Marshal Office, Director of Emergency Services, or Security Office. The contractor shall ensure compliance with all personal identity verification requirements as directed by Department of Defense (DoD), Headquarters Department of Army (HQDA) and/or local policy (see PWS 6.0). Should the Force Protection Condition (FPCON) change, the Government may require changes in contractor security matters or processes.

1.4.5.1. For Contractors Requiring Common Access Card (CAC). For installation(s)/location(s) cited in the contract, contractors shall ensure Common Access Cards (CACs) are obtained by all contract or subcontract personnel who meet one or both of the following criteria:

- a. Require logical access to Department of Defense computer networks and systems in either:
 - (i) the unclassified environment; or
 - (ii) the classified environment when authorized by governing security directives.
- b. Perform work, which requires the use of a CAC for installation entry control or physical access to facilities and buildings.
- c. While visiting or performing work on installation(s)/location(s), contractor personnel shall wear or prominently display the CAC as required by the Governing local policy.

1.4.5.1.1 Homeland Security Presidential Directive (HSPD)-12 Background Investigation Requirements: The contractor shall ensure that all contractor and subcontractor personnel whose duties require CAC card issuance obtain them and shall process all CAC card applications. The contractor shall ensure that all employees requiring an initial background investigation complete a Personnel Security Investigation Portal (PSIP) form upon hiring, and that this form is forwarded to the COR within 5 business days of hiring action. The COR will review the form for completeness and accuracy and forward it to the National Guard Bureau (NGB) Personnel Security manager who will initiate the investigation process via the PSIP. Contractor personnel will then receive two e-mail messages; the first will confirm that the investigative service provider has received the request, and

the second will provide instructions for the completion of the appropriate form via the Electronic Questionnaires for Investigations Processing (e-QIP) system. Upon completion of the e-QIP questionnaire and submittal of all required documents, including fingerprint card to the security manager, the background investigation (BI) will be initiated. The contractor shall ensure immediate compliance with all instructions regarding background investigation processing, including those provided verbally, by e-mail or via a Government system. The contractor is cautioned that the entire process from submittal of the PSIP form to return of the FBI fingerprint check may routinely take from two to six weeks and shall factor this lead time into its hiring/placement process. The contractor shall make all reasonable efforts to ensure that contractor employees meet CAC eligibility standards upon assignment to the contract and shall be held responsible for delays, failure to meet performance requirements or decreases in efficiency in accordance with the applicable inspection clause.

1.4.5.1.2 Trusted Associate Sponsorship System (TASS): The contractor shall process CAC applications through the TASS, the procedures for which are described below. Although there is no requirement for the contractor to designate a "Corporate Facility Security Officer" (FSO) to serve as its single point of contact for the BI, the TASS application process and other CAC and security-related matters, such designation facilitates these processes. If a Corporate FSO is not established, all contractor employees requiring a CAC will be required to process their own applications. The submission process for CAC applications is as follows:

- a. The contractor's FSO or contractor employee shall submit requests for a CAC via email to the designated TASS Trusted Agent (TA). The TASS TA for this requirement will be designated prior to award.
- b. The Government will establish a TASS application account for each CAC Request and will provide each contractor employee a user ID and password, via email, to the contractor's FSO or contractor employee. The FSO or contractor employee shall access the TASS account and complete the CAC application (entering/editing contractor information as applicable) at: <https://tass.dmdc.osd.mil/tass/>. The contractor's FSO or contractor employee shall follow up to ensure that the TA is processing the request.
- c. The Government will inform the contractor's applicant, via email, of one of the following:
 - (i) Approval. * Upon approval, the information is transferred to the DEERS database and an email notification is sent to the contractor with instructions on obtaining their CAC. The contractor proceeds to a RAPIDS station (RAPIDS Site Locator: <http://www.dmdc.osd.mil/rsl/>).
 - (ii) Rejection. * The Government, in separate correspondence, will provide reason(s) for rejection.
 - (iii) Return. Additional information or correction to the application required by the contractor employee.

*The contractor shall maintain records of all approved and rejected applications.

1.4.5.1.3 At the RAPIDS station, the RAPIDS Verification Officer will verify the contractor employee by Social Security Number (SSN) and two forms of identification. Identity source documents must come from the list of acceptable documents included in Form I-9, OMB No. 1615-0047, "Employment Eligibility Verification". Consistent with applicable law, at least one document from the Form I-9 list shall be a valid (unexpired) State or Federal Government-issued picture ID. The Identity documents will be inspected for authenticity and scanned and stored in the DEERS upon issuance of an ID. The photo ID requirement cannot be waived, consistent with applicable statutory requirements. The Verification Officer will capture primary and alternate fingerprints, picture, and updates to DEERS, and will then issue a CAC. Issued CACs will be valid for no longer than three years, or until the individual's contract end date (inclusive of any options), whichever is earlier.

1.4.5.1.4 The contractor shall manage requests for new or renewal CAC cards in sufficient time to ensure that all contractor employees have them when needed to perform work under this contract. The contractor shall provide at least 10 calendar days advance notice to the TA, unless there are extenuating circumstances approved by the COR or KO.

1.4.5.1.5 The contractor shall return issued CACs to the DEERS office upon departure or dismissal of each contractor employee, and shall obtain a receipt for each card and provide it to the TA/COR.

1.4.5.1.5.1 Failure to comply with these requirements may result in withholding of final payment

1.4.5.1.6. The contractor shall obtain an Army Knowledge Online (AKO) email address for each applicant, including subcontractors, who may be deployed or require logical access to a Government computer network. Note: If employees of a contractor lose the privilege to access AKO, they lose the ability to renew their CAC. Therefore, it is critical that contractor employees maintain their AKO accounts.

1.4.5.2 For contractors that do not require a CAC, but require access to a DoD facility or installation: Contractor and all associated sub-contractors employees shall comply with adjudication standards and procedures using the National Crime Information Center Interstate Identification (NCIC-III) and Terrorist Screening Database (TSDB) (Army Directive 2014-05), and applicable installation, facility and area commander installation/facility access, and local security policies and procedures (provided by a Government representative).

1.4.5.3 Awareness Training (AT) Level 1: All contractor employees, including subcontractor employees, requiring access to Army installations, facilities, and controlled access areas shall complete AT Level 1 training within 30 calendar days after contract start date and within 30 calendar days of new employees commencing performance. The contractor shall submit certificates of completion for each affected contractor and subcontractor employee to the COR within 15 calendar days after completion of training by each employee or subcontractor personnel. AT Level 1 awareness training is available at the following website: <https://jko.jten.mil/courses/at1/launch.html>.

1.4.5.4 iWATCH Training: The contractor and all associated sub-contractors shall brief all employees on the local iWATCH program (training standards provided by the requiring activity ATO). This local developed training will be used to inform employees of the types of behavior to watch for and instruct employees to report suspicious activity to the COR. This training shall be completed within 30

calendar days of contract award and within 30 calendar days of new employees commencing performance with the results reported to the COR NLT 30 calendar days after contract award.

1.4.5.5 Communications Security/Information Technology (COMSEC/IT) Security. All communications with DoD organizations are subject to COMSEC review. All telephone communications networks are continually subject to intercept by unfriendly intelligence organizations. DoD has authorized the military departments to conduct COMSEC monitoring and recording of telephone calls originating from, or terminating at, DoD organizations. Therefore, the contractor is advised that any time contractor personnel place or receive a call they are subject to COMSEC procedures. The contractor shall ensure wide and frequent dissemination of the above information to all employees dealing with DoD information. The contractor shall abide by all Government regulations concerning the authorized use of the Government's computer network, including the restriction against using the network to recruit Government personnel or advertise job openings.

1.4.5.6 Use of Government Information Systems (IS) and access to Government networks is a revocable privilege, not a right. Users are the foundation of the DoD strategy, and their actions affect the most vulnerable portion of the Army Enterprise Infostructure (AEI). Contractor employees shall have a favorable background investigation or hold a security clearance and access approvals commensurate with the level of information processed or available on the system. Contractor employees shall:

1.4.5.6.1 Comply with the command's Acceptable Use Policy (AUP) for Government owned IS and sign an AUP prior to or upon account activation.

1.4.5.6.2 Complete initial and/or annual Information Assurance (IA) training as defined in the IA Best Business Practices (BBP) training (https://atc.us.army.mil/iastar/docs/Training_BBp.pdf).

1.4.5.6.3 Mark and safeguard files, output products, and storage media per classification level and disseminate them only to individuals authorized to receive them with a valid need to know.

1.4.5.6.4 Protect IS and IS peripherals located in their respective areas in accordance with physical security and data protection requirements.

1.4.5.6.5 Practice safe network and Internet operating principles and take no actions that threaten the integrity of the system or network.

1.4.5.7 Army Training Certification Tracking System (ATCTS): All contractor employees with access to a Government information system shall be registered in ATCTS (<https://atc.us.army.mil/iastar/index.php>) at commencement of services, and shall successfully complete the DoD Information Assurance awareness training prior to access to the IS and then annually thereafter. (<https://iatraining.us.army.mil/>).

1.4.5.8 Information Assurance (IA) Training. All contractor employees and associated subcontractor employees shall complete the DoD IA Awareness Training before issuance of network access and annually thereafter.

1.4.5.9. Reserved

1.4.5.10 Protection of Personally Identifiable Information (PII). The contractor shall protect all PII encountered in the performance of services in accordance with Defense Federal Acquisition Regulation Supplement (DFARS) 224.103 Personally Identifiable Information and Department of Defense Directive (DoDD) 5400.11, Department of Defense Privacy Program, and DoD 5400.11-R. If a PII breach results from the contractor's violation of the aforementioned policies, the contractor shall bear all notification costs, call-center support costs, and credit monitoring service costs for all individuals whose PII has been compromised.

1.4.5.11 OPSEC Training: In accordance with AR 530-1, Operations Security, new contractor employees shall complete Level I OPSEC training within 30 calendar days of their reporting for duty and annually thereafter. The contractor shall submit certificates of completion for each contractor employee to the COR within 15 calendar days after completion of training. Level 1 OPSEC training is available at <https://securityawareness.usalearning.gov/opsec/index.htm>.

1.4.5.12 Reserved

1.4.5.13 Reserved

1.4.5.14 Reserved

1.4.6 Physical Security. The contractor shall safeguard all Government property provided for contractor use. At the close of each work period, Government facilities, equipment and materials shall be secured.

1.4.6.1 Key Control NOTE: All references to keys include key cards. The contractor shall establish and implement methods of ensuring that no keys/key cards issued by the Government are lost or misplaced or are used by unauthorized persons. No keys issued by the Government shall be duplicated. Such procedures shall include turn-in of any issued keys by personnel who no longer require access to locked areas. The contractor shall include procedures covering key control in the QCP.

1.4.6.1.1 The contractor shall immediately report any occurrences of lost or duplicated keys/key cards to the COR.

1.4.6.1.2 In the event keys, other than master keys, are lost or duplicated, the contractor shall, upon direction by the KO, re-key or replace the affected lock or locks; however, the Government, at its option, may replace the affected lock or locks or perform re-keying. When the Government replaces or re-keys the locks, the Government will deduct the total cost of lock replacement or re-keying from the monthly payment due the contractor. In the event a master key is lost or duplicated, the Government will replace all locks and keys for that system and will deduct the total cost from the monthly payment due the contractor.

1.4.6.1.3 The contractor shall prohibit the use of the Government issued keys/key cards by any persons other than the contractor's employees. The contractor shall prohibit the opening of locked areas by contractor employees to permit entrance of persons other than contractor employees engaged in the performance of services in those areas, or personnel authorized entrance by the KO.

1.4.6.2 Lock Combinations: The contractor shall establish and implement methods of ensuring that no lock combinations are revealed to unauthorized persons. The contractor shall ensure that lock combinations are changed when personnel having access to the combinations no longer have a need to know such combinations. These procedures shall be included in the contractor's QCP.

1.4.7 Special Qualifications: Medical services shall be performed by licensed medical professionals with oversight by the ARARNG, Office of the State Surgeon. Providers requiring licenses shall be licensed in accordance with Army Regulation 40-68, para. 4-8. Army Regulation 40-68, para 4-8 states, "... U.S. based non- personal services contracted providers, in all disciplines, will be licensed by the State or jurisdiction in which they are providing services". Dental services shall be performed by licensed medical professionals. All PHA events shall be performed by licensed and certified medical professionals. Contractor shall be held liable and is responsible for ensuring and verifying the appropriate level of Credentialing / Privileging (Licensure, Diploma, background checks, etc.) for all contract providers in support of any ARARNG event. The contractor shall ensure all employees possess all required licenses as referenced in this PWS for the performance of this contract. This does not include education or other qualifications for the position in which the contractor employee is performing, dress codes, or other information. At any time, government personnel may request verification of license. (NOTE: The Government does not provide training to contractors. Contractors must ensure that any personnel performing under a contract are fully trained, licensed, certified, and otherwise qualified to provide services.)

1.4.8 Post Award Conference/Periodic Progress Meetings: The contractor agrees to attend any post award conference convened by the KO in accordance with FAR 42.5. The KO, COR and other Government personnel, as appropriate, may meet periodically with the contractor to review the contractor's performance. At these meetings, the KO will apprise the contractor of how the Government views the contractor's performance and the contractor shall apprise the Government of problems, if any, being experienced. The contractor shall resolve outstanding issues raised by the Government. Contractor attendance at these meetings shall be at no additional cost to the Government.

1.4.9 Contract Manager (CM): The contractor shall designate a CM who shall ensure performance under this contract. The name of this person, and an alternate who shall act for the contractor when the CM is absent, shall be designated in writing to the KO. The CM or alternate shall have full authority to act for the contractor on all contract matters relating to daily operation of this contract. The CM shall work through the COR to resolve issues, receive technical instructions, and ensure adequate performance of services. The CM shall ensure that contractor employees do not perform any services outside the scope of the contract without an official modification issued by the KO. The CM shall ensure contractor employees understand that services performed outside the scope of the contract are performed wholly at the expense of the contractor.

1.4.10 Identification of Contractor Employees: All contractor personnel attending meetings, answering Government telephones, and working in other situations where their contractor status is

not obvious to third parties are required to identify themselves as such to avoid creating an impression that they are Government employees. The contractor shall ensure that all documents or reports produced by contractor personnel are suitably marked as contractor products or that contractor participation is appropriately disclosed. The contractor's status as a "contractor" shall be predominantly displayed in all correspondence types (to include signature blocks on e-mail) and dealings with Government or non-Government entities. Contractor personnel shall wear identification badges distinguishing themselves as such. The badges shall have the company name, employee name and the word "contractor" displayed.

1.4.10.1 The contractor shall retrieve all identification media (including vehicle passes) from its employees who depart employment for any reason. The contractor shall return all identification media (i.e., badges and vehicles pass) to the KO within 14 days of an employee's departure.

1.4.11. Combating Trafficking in Persons: The United States Government has adopted a zero-tolerance policy regarding trafficking in persons. Contractors and contractor employees shall not engage in severe forms of trafficking in persons during the period of performance of the contract; procure commercial sex acts during the period of performance of the contract; or use forced labor in the performance of the contract. The Contractor shall notify its employees of the United States Government's zero tolerance policy, the actions that will be taken against employees for violations of this policy. The contractor shall take appropriate action, up to and including termination, against employees or subcontractors that violate the US Government policy as described at FAR 22.17.

1.4.12 Reserved

1.4.13 Data Rights: The Government has unlimited rights to all documents/materials produced under this contract. All documents and materials, to include the source codes of any software, produced under this contract shall be Government owned and are the property of the Government with all rights and privileges of ownership/copyright belonging exclusively to the Government. These documents and materials may not be used or sold by the contractor without written permission from the KO. All materials supplied to the Government shall be the sole property of the Government and may not be used for any other purpose. This right does not abrogate any other Government rights.

1.4.14 Organizational Conflicts of Interest (OCI): The contractor and subcontractor personnel performing services under this contract may receive, have access to or participate in the development of proprietary or source selection information (e.g., cost or pricing information, budget information or analyses, specifications or work statements, etc.) or perform evaluation services which may create a current or subsequent OCIs, as defined in FAR Subpart 9.5. The contractor shall notify the KO immediately whenever it becomes aware that such access or participation may result in any actual or potential OCI and shall promptly submit a plan to the KO to avoid or mitigate any such OCI. The contractor's mitigation plan will be determined to be acceptable solely at the discretion of the KO. In the event the KO unilaterally determines that any such OCI cannot be satisfactorily avoided or mitigated, the KO may impose other remedies as he or she deems necessary, including prohibiting the contractor from participation in subsequent contracted requirements which may be affected by the OCI.

1.4.15 Reserved

2.0 Definitions and Acronyms:

2.1 Definitions:

2.1.1 Contractor: A supplier or vendor awarded a contract to provide specific supplies or service to the Government. The term used in this contract refers to the prime.

2.1.2 Defective Service: A service output that does not meet the standard of performance associated with the PWS.

2.1.3 Deliverable: Anything that can be physically delivered and includes non-manufactured things such as meeting minutes or reports.

2.1.4 Key Personnel: Contractor personnel that are evaluated in a source selection process and that may be required to be used in the performance of a contract by the PWS. When key personnel are used as an evaluation factor in best value procurement, an offer can be rejected if it does not have a firm commitment from the persons that are listed in the proposal.

2.1.5 Physical Security: Actions that prevent the loss or damage of Government property.

2.1.6 Quality Assurance: The Government procedures to verify that services being performed by the Contractor are performed according to acceptable standards.

2.1.7 Quality Assurance Surveillance Plan (QASP): An organized written document specifying the surveillance methodology to be used for surveillance of contractor performance.

2.1.8 Quality Control: All necessary measures taken by the Contractor to ensure that the quality of an end product or service shall meet contract requirements.

2.1.9 Subcontractor: One that enters into a contract with a prime contractor. The Government does not have privity of contract with the subcontractor.

2.1.10 Dental Classification (DENCLASS): An approved ARNG web-based module (through MEDCHART) designed to standardize the court of dental classifications, eliminate duplicate data system entry, facilitate the dental exam workflow, standardize the documentation of the health history and record of dental exam and integrate dental readiness within the Medical Operational Data Systems (MODS) architecture. The DENCLASS module has been designed to document the dental readiness requirements of all ARNG Soldiers. Components of DENCLASS include an automated health history, an automated SF603A, an automated letter outlining the dental treatment plan, a repository for dental radiographs, a library of Dental readiness policy and statistical reporting of all components of DENCLASS.

2.1.11 Individual Medical Readiness (IMR): is defined as being free of health-related conditions that limit a Soldier's ability to actively fulfill his/her assigned mission. Individual Medical Readiness consists of six elements: Dental Readiness, Immunization Status, Periodic Health Assessment (PHA), No deployment limiting conditions, Medical Readiness Laboratory Tests (HIV testing, DNA

sampling, G6PD test), and Individual Medical Equipment (Eyeglasses, hearing protection). When all of these factors are considered, the individual Soldier is classified as:

GREEN – Fully Medically Ready. Current in all categories including Dental Class 1 or 2.

AMBER – Partially Medically Ready. Lacking one or more immunizations, readiness laboratory studies, or individual medical equipment. Needs an element that can be corrected prior to mobilization.

RED – Not Medically Ready. Existence of a chronic or prolonged deployment limiting condition (per Service-specific physical standards guidelines), including Service members who are hospitalized or convalescing from serious illness or injury, or individuals in dental class 3. Has a limiting condition that requires hospitalization or required medical care not available in the theater of operation.

Medical readiness indeterminate. Inability to determine the Service member's current health status because of missing health information such as a lost medical record, an overdue PHA or being in dental class 4.

2.1.12 End of Event Summary: A summary that includes detailed information from the start of the event listing all numbers, classifications and types of services provided.

2.1.13 The Medical Operational Data System (MODS) (<http://www.mods.army.mil>) is a Medical Health Service Support objective system that provides the Army Medical Department (AMEDD) with an integrated automation system that supports all phases of Human Resource Life-Cycle Management in both peacetime and mobilization. This on-line system provides commanders, staffs, and functional managers of AMEDD organizations with a real time source of information on the qualifications, training, special pay and readiness of Army Medical Department (AMEDD) personnel. MODS currently maintain sixty system interfaces which are updated on a daily, weekly, monthly, or semi-annual basis. These interfaces bring in over 75% of the data used in the system. The AMEDD community has a powerful tool with which decision makers and managers can affect their work requirements quickly. As a desktop resource for the personnel manager, MODS places critical assignment, training, and qualification information at his fingertips. Time spent acquiring data can now be used in analyzing the information.

2.1.14 Medical Protection System (MEDPROS): The MEDPROS (available through MODS) was developed by the AMEDD to track all immunization, medical readiness and deploy ability data for all Active and Reserve components of the Army as well as DA Civilians, Contractors, and others. The comprehensive Medical Readiness data includes all medical and dental readiness requirements, IAW AR 600-8-101. They include immunizations, permanent physical profiles/duty limitations, eyeglasses/inserts, blood type, medical warning tags, personal deployment meds, pregnancy screening, DNA, HIV, and dental status among other data elements.

2.1.15 Periodic Health Assessment (PHA): An annual assessment for changes in health status, especially those that could impact a member 's ability to perform military duties.

2.2 Acronyms:

AAR	After Action Review
ADA	American Dental Association
ADCS	Army Dental Care System
ADDR	Active-Duty Dental Repository
AEI	Army Enterprise Infostructure
AMEDD	Army Medical Department
AR	Army Regulation
ARNG	Army National Guard
ARNP	Army Nurse Practitioner
AT/OPSEC	Antiterrorism/Operational Security
ASDRS	Army Selected Reserve Dental Readiness System
BI	Background Investigation
BMP	Basic Metabolic Panel
CAC	Common Access Card
CHEM 12	Complete Metabolic Panel
CLIA	Clinical Laboratory Improvement Amendments
CM	Contract Manager
COR	Contracting Officer Representative
DA	Department of the Army
DD254	Department of Defense Contract Security Classification Specification
DENCLASS	Dental Classification
DENCOM	US Army Dental Command
DFARS	Defense Federal Acquisition Regulation Supplement
DoD	Department of Defense
DRE	Direct Rectal Exam
DTR	Dental Treatment Record
FAR	Federal Acquisition Regulation
FDA	Food and Drug Administration
GFP/M/E/S	Government Furnished Property/Material/Equipment/Services
G6PD	Glucose 6 Phosphate Dehydrogenase
HCG	Human Chorionic Gonadotropin
HDL	High Density Lipids
HIPPA	Health Insurance Portability and Accountability Act
HQDA	Headquarters, Department of the Army
HRR	Health Readiness Record
HSPD	Homeland Security Presidential Directive
IA	Information Assurance
IAW	In Accordance With
IMR	Individual Medical Readiness
IS	Information System(s)
IT	Information Technology
KO	Contracting Officer
LDL	Low Density Lipids
MEDCHART	Medical Electronic Data Care History And Readiness Tracking
MEDPROS	Medical Protection System
MHA	Medical Health Assessments

MODS	Medical Operational Data System
NCOIC	Non-Commissioned Officer In Charge
MRC	Medical Readiness Category
NGB	National Guard Bureau
NOSTRA	Naval Ophthalmic Support and Training Activity
NP	Nurse Practitioner
OCI	Organizational Conflict of Interest
OD	Doctor of Optometry
OSS	Office of the State Surgeon
OSHA	Occupational Safety and Health Administration
PHA	Periodic Health Assessment
PII	Personally Identifiable Information
PIPO	Phase In/Phase Out
POC	Point of Contact
POS	Point of Service
PRS	Performance Requirements Summary
PSR	Periodontal Screening and Repair
PWS	Performance Work Statement
QA	Quality Assurance
QASP	Quality Assurance Surveillance Plan
QC	Quality Control
QCP	Quality Control Program
RDA	Registered Dental Assistant
RPRa	Rapid Plasma Realign
SELRES	Army Selective Reserve
SRM	Soldier Readiness Model
SMRC	Soldier Medical Readiness Center
SRC	Soldier Readiness Check
SRP	Soldier Readiness Processing
SRTS	Spectacle Request Transmission System
SSN	Social Security Number
STR	Soldier Treatment Record
TE	Technical Exhibit
TGS	Triglycerides
ARARNG	Arkansas Army National Guard
USD(I)	Under Secretary of Defense for Intelligence
USPSTF	US Preventive Services Task Force

3.0 Government Furnished Property, Material, Equipment and Services (GFP/M/E/S): The Government will provide the property, material, equipment, and/or services listed below solely for the purpose of performance under this contract:

3.1 Property: The Government shall provide parking for HIPAA, Privacy Act, OSHA & Radiological-Compliant, mobile platform, Medical/Dental Practices. Contractor will have the ability to perform all ordered services to operate the event with only parking space provided by Government. Contractor will provide mobile platforms.

3.1.1 Armory, SMRC or office space will be utilized when authorized per event by the government with correspondence with COR in advance of the start date of the event. When Government space is utilized, contractor will be required to provide all necessary support equipment, i.e., tables, chairs, privacy dividers, extension cords, power strips, internet access, computers etc. The contractor understand that no space will be available unless otherwise told by the COR in advance of the event.

3.2 Reserved

3.3 Reserved

3.4 Reserved

3.5 Utilities: All utilities in the facility will be available for the contractor's use in the performance of this contract. The contractor shall instruct employees in utilities conservation practices. The contractor shall operate under conditions that preclude the waste of utilities, which include turning off the water faucets or valves after using the required amount.

4.0 Contractor Furnished Property, Materials, and Equipment (CFP/M/E):

4.1 General: Except for those items specifically stated to be Government-Furnished in Paragraph 3.0, the contractor shall furnish everything required to perform these services as indicated in Paragraph 1.1.

4.2 Reserved

4.3 Reserved

5.0 Requirements: The contractor shall:

5.1. Contractor shall provide all personnel, equipment, tools, materials, supervision, and quality control needed to perform the IMR requirements. In accordance with Army Regulation 40-68, para. 4-8, contracted providers, in all disciplines, must be licensed by the State of Arkansas. When hiring personnel, the contractor shall keep in mind that the stability and continuity of the work force are essential.

5.1.1. Contractor shall provide health readiness administrator and team to service all case management records and administrative requirements for all scheduled medical events.

5.1.2. Contractor shall provide management, execution direction, and oversight of the contracted employees to include directing staff, updating health records, case management and employee relations when questions or issues arise during the event.

5.1.3. Contractor shall provide in-person or via help-desk IT technicians to service on- spot for electronic hardware, software related to the events.

5.2 Mobile Dental and Medical Platforms shall be IAW Occupational Safety and the Health Act of 1970 (Part 1910), Subpart E Regulations Standards 29 CFR. Mobile Dental and Medical Platforms shall meet all OSHA, ADA, Privacy Act & HIPAA standards to ensure a safe and productive workspace for medical services under the Individual Readiness Requirements. Mobile Dental and Medical Platforms shall be a fully functional workspace in the event no additional space is provided inside military buildings during events, as these events may be in remote areas such as artillery ranges. These platforms shall provide protection from the weather elements and provide climate control.

5.2.1 The Mobile Dental and Medical Platforms shall provide radiograph radiation protection, digital bitewing and panoramic equipment to provide digital radiography files at the end of the event, all emergency medical resources consistent with OSHA such as eye-wash station, distilled water tanks, separate and walled dental operatories to provide confidentiality for patients and individual temperature-controlled rooms consistent with OSHA standards. The Contractor's Safety Program shall be IAW OSHA Standards Emergency Action Plans, (1910), Subpart E, e-CFR, Means of Egress. The Contractor 's safety program includes equipment training, occupational safety, health, and fire protection guidance to ensure the protection and health of all work personnel.

5.2.2 Mobile Dental and Medical Platforms shall include and be capable of providing the following:

5.2.2.1 Fully equipped Private Exam Rooms for full PHA & Physical Exam Requirements. (Cannot be an "open-bay" examination room setup; shall be HIPAA-Compliant & confidential for patient)

5.2.2.2 Health Stations to include vital signs, Visual acuity, optometry services, tonometry, DRE (Digital Rectal Exam), Hemocult, immunizations, PPD screenings, Sickie Cell Trait (SCT) screenings, HIV Blood-Draws, DNA collection, DOEHRs –Compliant audiometry, and all other MEDICAL/DENTAL READINESS requirements.

5.2.2.3 Lab with eye-wash stations.

5.2.2.4 Biohazard separated work areas.

5.2.2.5 A sufficient size reception area with full office capabilities to accommodate continuous flow of patient services.

5.2.2.6 Have appropriate climate control and ventilation system to reduce airborne pathogens.

5.2.2.7 DOEHRs Compatible Audiometer.

5.2.2.8 TITMUS Visual Acuity Screener.

5.2.2.9 CLIA – Waived Rapid Laboratory Screenings (Lipid Profile to include total cholesterol, HDL, LDL, TGS, blood glucose, BMP, CHEM 12, Hemoglobin A1C, Urinalysis & Urine HCG) and urine Chlamydia/Gonorrhea screen.

5.2.2.10 Blood Pressure, Pulse Oximetry, Height/Weight/Temperature.

5.2.2.11 Optometry with Visual Refraction.

5.2.2.12 Ability to Perform Blood Collection for HIV, DNA, G6PD, RPRa, SCT. Contractor is not required to provide HIV or DNA supplies or shipping materials. Arkansas ARNG Medical Detachment will provide all HIV and DNA supplies, shipping supplies, and shipping guidance. Contractor shall drop off packaged labs to the local commercial shipping company/service.

5.2.2.13 Ability to Perform and Administer Immunizations - Contractor shall provide immunization supplies, (needles, syringes, alcohol pads, band aids, etc.). TMRD will provide all required immunizations for each event.

5.2.2.14 There is current federal mobilization requirement to conduct Women's Health Exams such as Pap smears, Chlamydia testing, mammograms as required per Women's Health guidance, and case manage Mammography. The contractor shall be prepared to conduct these IAW current US Preventive Health Task Force once given at least 60 days notice by the COR.

5.2.2.15 There is current federal mobilization requirement to conduct DRE (Digital Rectal Exam) and PSA testing for over fifty, male service members. The ability to perform DRE (Digital Rectal Exam) and PSA testing for over fifty, male service members shall be provided IAW current US Preventive Health Task Force once given at least 60 days notice by the COR.

5.2.2.16 Ability to upload audiograms directly into the DOEHRS Repository.

5.2.2.17 Contractor is responsible to keep and dispose of all medical waste in accordance with 42 CFR.

5.2.2.18 Ability to provide Tonometry.

5.2.2.19 Ability to provide EKG.

5.2.2.20 Ability to provide and perform Pulmonary Function Testing

5.2.2.21 Operatory (Private Exam Room) Radiological-Compliant & Self- Contained mobile platform Dental Practice shall contain/provide:

5.2.2.22 Fully digital Intra-Oral Bite-wing radiographs, Split-Bitewing capable Panoramic X-ray Machine & examinations, provide radiograph radiation protection.

5.2.2.23 Professional grade dental treatment comparable to a fixed-site, traditional dental practice.

5.2.2.24 Professional-grade dental vacuum and compressors to allow general & specialty dental procedures.

5.2.2.25 Ergonomically correct, fully functional dental exam & treatment chairs.

5.2.2.26 Reception area for data intake and outtake within mobile platform Dental Practice for health history and data processing.

5.2.2.27 Reception area shall also have patient education capability, i.e. To Quit Smoking-Smoking Sensation.

5.2.2.28 Meet all state radiological requirements, Arkansas Health and Human Services Regulatory Guide 4.4 Dental Facilities.

5.2.2.29 Separate sterilization area within the mobile platform Dental Practice

5.2.2.30 Fully functional office area that has full printing/copying requirements.

5.2.2.31 Equipped with printers that provide Film-Based printing for superior clinical quality and durability of hard copy x-ray.

5.2.2.32 Shall contain biohazard waste receptacles.

5.2.2.33 Intra-Oral Bitewing Sensors for superior digital image exposure, Kodak preferred.

5.2.2.34 Equipped to provide SMs with patient education and shall have a Dental Quality Assurance program/consultant with Military Dental Experience.

5.2.2.35 Portable Dental exam/radiograph equipment shall meet all State radiological standards and requirements.

5.2.3 All vehicles shall be radiological compliant and have sufficient patient walkways to provide comfortable space during heavy utilization events. Internet connectivity shall be available for any mobile platform Health Practice deployed to allow for real-time data entry at events.

5.2.4 Mobile Health Practice shall have capability for real-time, point of service military health records upload. This requires Contractor to have high-speed, broadband or satellite communication channel. The contractor shall have Project Manager and CAC enabled Health Care Providers with Medical Health Assessments access to update Soldier PHA's in real time. The workstations shall have Bar-Code Scanning capability to ensure high quality standards for data entry. Contractor shall be able to do post-Operatory digital Panoramic X-rays upon completion of treatment whereby the forensics image shall change materially. Contractor shall be able to upload images to the Active-Duty Dental Repository (ADDRs) system for Dental Command requirements of deploy ability. Contractor shall have live event & Soldier tracking systems in place to report progress of events back to Point-of-Contact at Event as well as for remote tracking.

5.2.5 Setup of Contractor 's dental equipment shall be completed by the day prior to event commencement. Setup and breakdown of Contractor 's dental equipment is the sole responsibility of the Contractor.

5.2.6 All vendor property to include mobile platforms needs to be removed within a 24-hour period upon completion of the event. In the case of an emergency such as hurricanes etc. the vendor shall have capability to completely move off the Government site within 5 hours of notice.

5.2.7 Contractor shall notify the COR and event POC via e-mail at the conclusion of each day's event operations with a summary report of the total number of Soldiers scheduled, checked-in, and checked-out for that day.

5.3 Digital Bitewings, Digital Panoramic Radiographs & Dental Exams: The Contractor shall perform required dental examinations to include digital bitewings and digital panoramic radiographs if clinically indicated in support of the annual dental screenings in accordance with the AR 40-501 and AR 40-35. The Contractor shall upload all exam and digital radiographs into the DENCLASS system. The Contractor shall ensure within five days post event all Soldiers have a current Panoramic Radiograph on file in accordance with NGB standards and if missing, the Contractor shall perform new radiographs. Adherence to all OSHA/HIPAA/ADA/PRIVACY ACT requirements, State and Federal Regulations are mandatory. Film or digitized radiographs shall not be accepted.

5.3.1 Exam - A periodic oral evaluation shall assess the current state of oral health, risk for future dental disease as well as assessing general health factors that relate to the treatment of the patient. Personnel performing the dental screening examination have a moral and ethical obligation to inform the Soldier if they observe or are apprised of any sign or symptom for which the Soldier should obtain further evaluation or care. The following measures are required for each periodic oral evaluation:

5.3.1.1 Caries risk assessment – Classify Soldiers as low, moderate, or high risk for caries as outlined by the American Dental Association 's 1995 Special Supplement. This assessment shall allow the ADCS to identify those at high risk for future disease and appropriately manage their care.

5.3.1.2 Periodontal assessment – Periodontal Screening and Recording (PSR), a screening procedure endorsed by the American Dental Association and the American Academy of Periodontology, should be utilized to determine the need for periodontal treatment.

5.3.1.3 Tobacco risk assessment – Classify Soldiers as a smoker, user of smokeless tobacco or as both.

5.3.1.4 Oral cancer screening – Perform a thorough oral cancer screening IAW American Dental Association.

5.3.1.5 Digital radiographs should be taken only for clinical reasons according to FDA guidelines as determined by the patient 's dentist. When required, Radiographs shall be of diagnostic quality, properly identified and dated. A panograph is required to be present in dental record and of adequate quality for diagnostic/identification purposes. There is no time requirement on updating panographic radiographs. However, the panographic radiograph shall adequately represent the current oral condition of the Soldier. Digital panograph shall be performed on all service members who do not have a panograph on file in the DENCLASS system, regardless of when the hardcopy panograph is dated.

5.3.1.6 Provide hardcopy medical documentation review, file, and organization in accordance with AR 40-66 into the Soldier Treatment Record (STR). Thermal printers printing film radiographs are preferred.

5.3.1.7 Each exam shall be entered into the DENCLASS reporting system and annotated on the SF 603 (Health Record – Dental) and the SF 5570 (Health Questionnaire for Dental Treatment) of the Soldier's dental records.

5.3.1.8 Immediately provide feedback and work progress/ status of number of Dental examinations completed to the Office of the State Surgeon 's POC (or designated representative) when it is determined that timelines or other factors may negatively impact processing 8 Soldier exams per hour.

5.3.1.9 Electronic entry of data into the DENCLASS database is required within 5 days of completion of the Dental Screening event to include the upload of any radiographs taken during the event.

5.4 Dental Treatment: The Contractor shall provide approved dental treatment only within an OSHA & ADA Compliant mobile platform Dental Practice. The goal is for the contractor to convert 90% from Dental Classification 3 to Dental Classification 1 or 2. Soldiers not converted should fall IAW PWS reference.

5.4.1. Contractor shall have fully licensed & accredited in the state that the event is scheduled that can treat authorized Class 3 ARARNG Service Members. The vast majority of events will be held in Arkansas. See Technical Exhibit 4 for Dental Classification Information and Technical Exhibit 5 for Authorized and Unauthorized Procedures.

5.4.2 Contractor shall submit all dental treatment plans to the State Dental Officer (or designated representative) for approval prior to treatment in DENCLASS. Treatment exceptions can be made, but they shall be coordinated with the State Surgeon.

5.4.3 Post-event Dental Case Management:

5.4.3.1 Post-event dental case management is not to exceed 30 days from the end of the event for Soldiers whose dental status cannot be converted to DRC 1 or 2 on-site. Soldiers requiring post-event dental case management shall be case managed by the Contractor based upon an approved dental treatment plan. The Contractor shall arrange for fulfillment of dental treatment for any Soldier requiring these services. Within 24 hours of completion of the event, the Contractor shall provide a by-name roster of all Soldiers that require post-event dental case management to the Office of the State Surgeon. Contractor shall provide a by-name update upon conversion to DRC 1 or 2. Upon completion of the initial 30 days of post-event dental case management, the Contractor shall invoice for dental case management services provided.

5.4.3.2 Contractor shall setup a network of and account with local general and specialty fixed-site dental facilities and offices near event location and/or home of ARARNG Soldier requiring treatment.

5.4.3.3 Upon need of dentally treated Soldier and recommended by dentist, Contractor shall arrange for fulfillment of dental treatment for Soldier.

5.4.3.4 Contractor shall explain post-operative instructions or verify that instructions were provided by treating Dentist to ARARNG treated Soldier and any Supervisor if necessary. Follow-up

conversations with ARARNG treated Soldier and Supervisor may be necessary depending on complexity of treatment and, if any, prescription medication for post-operative.

5.4.3.5 Post-event dental case management exceeding 30 days: Soldier's whose treatment will extend beyond 30 days post-event shall require direct coordination with the Office of the State Surgeon to determine appropriate course of action based upon treatment plan.

5.5 Prescription Medication Services:

5.5.1 Contractor shall setup network & account with local pharmacies near event location.

5.5.2 Upon need of dentally treated Soldiers and when recommended by dentist, Contractor shall arrange for fulfillment of prescription medication for Soldier.

5.5.3 Contractor shall explain prescription medication instructions to ARARNG treated Soldier and any other POC as necessary or verify that instructions were provided by the local pharmacy. Follow-up conversations between the ARARNG treated Soldier and Supervisor may be necessary depending on complexity of treatment and prescription medication.

5.5.4 Contractor shall provide all prescription medication for soldiers undergoing dental treatment (analgesia and antibiotics).

5.6 Periodic Health Assessments (PHA) and Physical Exams: The Contractor shall conduct PHAs in accordance with the PHA guidance set forth by the National Guard Bureau Office of the Chief Surgeon and other physical exams, to include: Flight; Airborne, Ranger, and Special Forces training and duty in accordance with AR 40-501. The Contractor shall conduct physicals utilizing a combination of a HIPAA, Privacy Act, OSHA & ADA Compliant mobile platform/ Portable Health Vehicle, and portable equipment to be used inside an Armory. Equipment is required to conduct visual acuity, ANSI certified and DOEHRs compatible audiometry, on-site laboratory testing for lipids, glucose, urinalysis, pregnancy testing, and hemoglobin/hematocrit, a complete physical exam by a certified clinician, tonometry, and vital signs. Contractor shall conduct atherosclerotic cardiovascular disease screening for all personnel over the age of 40 IAW US Preventive Services Task Force. All PHAs will be performed in the MHA module of MODs and completed the day of the event.

5.6.1 Vision Exams:

5.6.1.1 Visual Acuity Testing: The Contractor shall conduct visual acuity testing in accordance with AR 40-501. The Contractor shall schedule all vision services in conjunction with other services.

5.6.1.2 The Contractor shall provide vision screening to include a Snellen test for all SMs, and screening to determine near and far vision measurements, refractions, pupillary distance for SMs requiring prescription lenses. The Contractor shall ensure the Doctor of Optometry fully completes the DD Form 771.

5.6.1.3 Contractor shall have the capability to conduct complete optometry exams. The contractor shall use Spectacle Request Transmission System (SRTS) for placing prescription orders thru

NOSTRA. The Contractor shall transfer current existing prescriptions to DD Form 771. As required on each task order the contractor shall ensure the five pair of standard issue eye wear are ordered in NOSTRA: 1) MS9 clear; 2) MS9 sun; 3) M40 (mask Insert); 4) FOC (Frame of choice), 5) ESS ICE (ballistics eyewear) (or current requirement) as determined by the task order. All eye wear shall have secondary prescription validation, be sorted by Soldier and unit, and sent directly to the Unit Representative for issue within seven (7) days of receipt from NOSTRA. The Contractor shall utilize the current accepted tracking shipping method.

5.6.1.4 Contractor shall scan and index all DD Form 771 into HRR IAW AR 40-66 within seven (7) days of event. Hard copies of all DD Form 771 prescriptions shall be provided to the ARARNG Medical Detachment for filing in STR within five (5) days of event.

5.6.1.5. Visual screening results shall be documented in MEDPROS within 5 days of receiving results. Each SM shall be assigned a Vision Readiness Classification according to AR 40-501.

5.6.2 DOEHRS (Defense Occupations and Environmental Health Readiness System) Compatible Audiometer Testing: The Contractor shall conduct audiometric evaluations in accordance with AR-501. Audiometric evaluations shall be DOEHRS compatible. Adherence to all requirements of OSHA, State and Federal Regulations is mandatory. Military Occupational Hearing Test (MOHT) testing shall be performed IAW all applicable OSHA, State and Federal guidelines. Contractor shall have Audiologist on site of the event. 100% of the results of hearing test shall be entered into DOEHRS within five (5) days of the event. Hearing test shall be conducted utilizing a mobile platform Hearing Practice. The Contractor shall ensure effectiveness and accuracy of tests and create an environment most suited to limit noise pollution that could affect testing. If MOHT test is required and cannot be done on site, Contractor shall case manage for hearing services within 30 days. It is highly preferred to perform all testing on site.

5.6.3 Phlebotomy / Lab: The Contractor shall conduct all required CLIA-waived laboratory services on-site during the event. Non CLIA waived laboratory requirements will be shipped in accordance with the ARARNG Medical Detachment shipping standards. ARARNG shall provide all supplies, shipping materials, and shipping labels associated with HIV testing and DNA Collection. All HIV and DNA specimens shall be shipped by the Contractor to the approved DA testing facility. The Contractor shall utilize the current accepted shipping method coordinated with ARARNG Medical Detachment. All other laboratory supplies/testing is the responsibility of the Contractor.

CONTRACTOR WILL NOTIFY THE COR OF LABORATORY VALUES OUTSIDE OF NORMAL RANGE VIA E-MAIL IMMEDIATELY UPON RECEIPT. Contractor shall scan and index all laboratory results into HRR IAW AR 40-66. Hard copies of all laboratory work shall be provided to ARARNG Medical Detachment within seven (7) days of the draw for filing in the STR. Contractor shall update MEDPROS with required laboratory results within five (5) days of receiving results.

5.6.4 Sick Cell Trait (SCT) Screenings during patient encounters at all medical events will begin reviewing Soldier's records to verify Soldiers have a SCT screening annotated in MEDPROS and Electronic Medical Record (EMR) 5 days post event.

5.6.4.1 Soldier's with a positive SCT screening will be issued a red medical warning tag IAW AR 40-66 upon notification of positive screening results.

5.7 The Contractor shall provide access to Project Managers before and after the event to ensure proper coordination and follow up. The Contractor shall provide contact information for all Event Managers to the COR and event POC no later than 5 days prior to the event. All Event Managers shall possess a Common Access Card (CAC) and have received training in MEDPROS, E-PHA, E-Profile, DENCLASS and HIPAA (Certifications of training required when applicable). The contractor shall follow the procedures stated in paragraph 1.4.5.1 for obtaining CACs for their employees.

5.8 The Contractor shall have a plan of action in the event of any equipment malfunction during events. Contractor shall need to be operational within 4 hours of malfunction. This time shall include troubleshooting, removing, and replacing equipment.

5.9 Contractor shall provide an End of Event Summary Report to the OFFICE OF THE STATE SURGEON within 5 days post event. The report shall be for all services provided, in accordance with HIPAA guidance, to include the following. The report shall be approved and verified by the ARARNG liaison with the office of the State Surgeon of the ARARNG. The report should consist, at a minimum, of the items listed below.

- Date
- Location
- List of contract employees (providers, technicians, and administrative staff)
- Total number of patients scheduled.
- Total number of patients examined.
- Total number of Soldiers by dental classification: 1, 2, 3
- Total number of bitewing radiographs (sets of 4 per patient) taken
- Total number of panograph radiographs taken
- Total number of PHAs (broken down by clinical services)
- Total number of Hearing Examinations & MOHT test
- Total number of immunizations by type administered.
- Verification of entry into MEDPROS & DENCLASS, E-PHA, E-Profile, HRR
- A by name listing of all Soldiers examined and the dental classification of the Soldier
- A by name listing of all Soldiers requiring post event Dental case management
- A by name listing of all Soldiers requiring Behavioral Health case management
- A by name listing of all Soldiers requiring medical case management for temporary/permanent profile determination

5.10 Service Contract Reporting. RESERVED

6.0 Publications: Publications applicable to this PWS are listed below:

Publication (Chapter/Page)	Date of Publication	Mandatory or Advisory	Website
Federal Acquisition Regulation			https://www.acquisition.gov/?q=browsefar
Defense Federal Acquisition Regulation Supplement			http://www.acq.osd.mil/dpap/dars/dfarspgi/current/index.html or https://www.acquisition.gov/dfars
Joint Travel Regulation (JTR)			https://www.defensetravel.dod.mil/site/travelreg.cfm
DoDM 1000.13-M-V1 DoD Identification (ID) Cards	01/23/2014		http://www.esd.whs.mil/Directives/issuances/dodm

(Enclosure 2, paragraph 3.b)	(Change 1: 07/28/2020)		
Federal Information Processing Standards (FIPS) Publication 201-2 Personal Identity Verification (PIV) of Federal Employees and Contractors (paragraph 9)	August 2013		http://nvlpubs.nist.gov/nistpubs/FIPS/NIST.FIPS.201-2.pdf
DoDM 5200.02 Procedures for the DoD Personnel Security Program (PSP)	04/03/2017		https://www.esd.whs.mil/Directives/issuances/dodm/
DoDI 5200.46 DoD Investigative and Adjudicative Guidance for Issuing the Common Access Card (CAC)	09/09/14 (Change 2: 2 NOV 2020)		https://www.esd.whs.mil/Directives/issuances/dodi/
Homeland Security Presidential Directive (HSPD)-12 Policy for a Common Identification Standard for Federal Employees and Contractors	27 MAR 2024		https://www.dhs.gov/homeland-security-presidential-directive-12
DoDI 5400.11 Department of Defense Privacy and Civil Liberties Programs	01/29/2019		https://www.esd.whs.mil/Directives/issuances/dodi/
DoD 5400.11-R Department of Defense Privacy Program	05/14/2007		https://www.esd.whs.mil/Directives/issuances/dodm/
DoDD 8140.01 Cyberspace Workforce Management	10/05/2020		https://www.esd.whs.mil/Directives/issuances/dodd/
DoDM 8140.03 Cyberspace Workforce Qualification and Management Program	02/15/2023		https://www.esd.whs.mil/Directives/issuances/dodm/
DoD 5220.22-M National Industrial Security Program Operating Manual (NISPOM)	18 MAR 2011 (Change 2: 24 SEP 2020)		https://www.esd.whs.mil/Directives/issuances/dodm/
Army Directive 2014-05 Policy and Implementation Procedures for Common Access Card Credentialing and Installation Access for Uncleared Contractors	03/07/2014		https://armypubs.army.mil/ProductMaps/PubForm/ArmyDir.aspx
AR 25-2 Information Assurance	04/04/2019		http://armypubs.army.mil/ProductMaps/PubForm/AR.aspx
AR 530-1 Operations Security	09/26/2014		http://armypubs.army.mil/ProductMaps/PubForm/AR.aspx
AR 525-13 Antiterrorism	12/3/2019		http://armypubs.armu.mil/ProductMaps/PubForm/AR.aspx
AR 381-12 Threat Awareness and Reporting Program (TARP) (Section II, ¶ 2-4.b)	06/01/2016		http://armypubs.army.mil/ProductMaps/PubForm/AR.aspx
HA Policy 02-11 Health affairs Clear and Legible Report Process	02/11/2011		https://health.mil/Reference-Center/Policies/2011/02/04/Clear-and-Legible-Report-Process
AR 40-66 Medical Record Administration and Healthcare Documentation	01/04/2010		https://armypubs.army.mil/epubs/DR_pubs/DR_a/pdf/web/r40_66.pdf
AR 40-68 CLinical Quality Management	05/22/2009		https://armypubs.army.mil/epubs/DR_pubs/DR_a/pdf/web/r40_68.pdf

AR 40-501 Standards of Medical Fitness	06/27/2019		https://armypubs.army.mil/epubs/DR_pubs/DR_a/ARN37720-AR_40-501-002-WEB-4.pdf
Occupational Safety and Health Act of 1970	01/01/2004 (Amended)		https://www.osha.gov/laws-regs/oshact/completeoshact

6.1 Applicable Forms: The Contractor shall ensure that all appropriate DA or DD forms (in accordance with references) are utilized for each service supplied. The forms shall be made available through the Arkansas Medical Detachment office. The Contractor shall provide all documentation concerning dental & medical treatment to the ARARNG. Forms applicable to the PWS are listed below:

Form	Date	Website
DD 1172-2 Application for Identification Card/DEERS Enrollment	04/24/2020	https://www.esd.whs.mil/Directives/forms/dd1000_1499/
I-9 Employment Eligibility Verification	08/01/2023	https://www.uscis.gov/sites/default/files/files/form/i-9.pdf
DD 441 Department of Defense Security Agreement	03/01/2020	https://www.esd.whs.mil/Directives/forms/dd0001_0499/
DD 250 Material Inspection and Receiving Report	08/01/2000	https://www.esd.whs.mil/Directives/forms/dd0001_0499/
DD771	06/01/1996	https://www.esd.whs.mil/Portals/54/Documents/DD/forms/dd/dd0771.pdf

TECHNICAL EXHIBIT 1
Performance Requirements Summary (PRS)

Performance Objective	Performance Standard	Acceptable Quality Levels (AQL)	Surveillance Method / By Whom
1.4.7, 5.1, 5.4.1 Licensure Requirement	Contracted providers, in all disciplines, shall be appropriately licensed or certified by their governing body.	100% of the time	100% Inspection / COR & OSS
5.2 through 5.2.6 Mobile Medical and Dental Platforms Requirement	Meet OSHA, ADA, Privacy Act and HIPPA standards to provide a safe and productive workspace. IAW Occupational Safety and the Health Act of 1970 (Part 1910), Regulations Standards, 29 CFR. IAW with Contractor's Safety Program, Emergency Action Plans, (1910), Subpart E, e-CFR, Means of Egress.	100% of the time	100% Inspection / COR & OSS
5.3.1.7 Data Entry to DENCLASS	Electronic entry of data into the DENCLASS database is required within 5 days of completion of the Dental Screening event to include the upload of any radiographs taken during the event.	No more than three instances per event where data is loaded later than 5 days after event completion	Random monitoring/ COR & OSS
5.4.3.1 Post-event dental case management	Within 24 hours of completion of the event, the Contractor shall provide a by-name roster of all Soldiers that require post-event dental case management to the Office of the State Surgeon	No more than one instance per period of performance of roster being submitted later than 24 hours after event	Random monitoring/ COR & OSS
5.8 Equipment Malfunction Plan	Contractor needs to be operational within 4 hours of a malfunction.	100% of the time	Random monitoring/ COR & OSS

TECHNICAL EXHIBIT 2
Deliverables Schedule

PWS Reference / Deliverable Title	Frequency	Number of Copies	Medium/Format	Submit To
1.4.3 Quality Control Plan	NLT 10 days after contract award	1	Electronic Submission	COR
1.4.5.3 Awareness Training (AT) Level 1 Certificates	Provide to COR within 15 calendar days after employee completes training.	1	Electronic Submission	COR
1.4.5.4 iWATCH Training	NLT 30 days after contract award or when new employee onboarded	1	Electronic Submission	COR
1.4.5.1 Background Investigation Requirements	NLT 5 days after employee on boarding	1	Electronic Submission	COR
1.4.5.11 OPSEC Training Certificates	Provide to COR within 15 calendar days after employee completes training.	1	Electronic Submission	COR
5.2.7 End of Day Summary Report of event operations. Total number of Soldier's scheduled, checked in and checked out.	End of Day of each event	1	Electronic Submission	COR
5.3.1.9 Dental Services 100% upload into DENCLASS	5 days post event	1	Electronic Submission	COR/Event POC
5.6.2 DOEHRS Input 100%	5 days post event	1	Electronic Submission	COR
5.6.4 Lab Results and Update in MEDPROS	Copy of results required to OSS within 7 days of draw & MEDPROS update within 5 days of receipt of results	1	Electronic Submission	COR
5.6.4.1 Sickle Cell Trait Screening (SCT)	Sickle Cell Trait (SCT) Screenings during patient encounters at all medical events will	1	Electronic Submission	COR

	begin reviewing Soldier's records to verify Soldiers have a SCT screening annotated in MEDPROS and Electronic Medical Record (EMR) 5 days post event, effectively immediately.			
5.6.4.2 Positive SCT Result	Soldiers with a positive SCT screening will be issued a red medical warning tag IAW AR 40-66 upon notification of positive screening results	1	In-Person	COR
5.7 Event Manager Contact Information	5 days prior to the event	1	Electronic Submission	COR
5.9 Event Summary	5 days post event	1	Electronic Submission	COR

TECHNICAL EXHIBIT 3

Estimated Workload Data

LOCATION	PAX (approx.)
North Little Rock (Camp Robinson)	250

TECHNICAL EXHIBIT 4

Dental Classification System

Class 1: Patients with a current dental examination, who do not require dental treatment or reevaluation. Class 1 patients are worldwide deployable.

Class 2: Patients with a current dental examination, which require non-urgent dental treatment or reevaluation for oral conditions, which are unlikely to result in dental emergencies within 12 months. Class 2 patients are worldwide deployable. Patients in dental class 2 may exhibit the following:

- a. Treatment or follow-up indicated for dental caries or minor defective restorations that can be maintained by the patient.
- b. Interim restorations or prosthesis that can be maintained for a 12 month period. This includes teeth that have been restored with permanent restorative materials for which protective cuspal coverage is indicated.
- c. Edentulous areas requiring prostheses but not on an immediate basis.
- d. Periodontium that:
 - 1) Requires oral prophylaxis.
 - 2) Requires maintenance therapy
 - 3) Requires treatment for slight to moderate periodontitis and stable cases of more advanced periodontitis
- e. Unerupted partially erupted or malposed teeth that are without historical, clinical or radiographic signs or symptoms of pathosis, but which are recommended for prophylactic removal.
- f. Active orthodontic treatment. The provider should consider placing the patient in passive appliances for deployment up to six months. For longer

periods of deployment, the provider should consider removing active appliances and placing the patient in passive retention.

- g. Temporomandibular disorder patients in remission. The provider anticipates the patient can perform duties while deployed without ongoing care and any medications or appliances required for maintenance shall not interfere with duties.

Class 3: Patients who require urgent or emergent dental treatment. Class 3 patients normally are not considered to be worldwide deployable.

- a. Treatment or follow-up indicated for dental caries, symptomatic tooth fracture or defective restorations that cannot be maintained by the patient.
- b. Interim restorations or prostheses that cannot be maintained for a 12-month period.
- c. Patients requiring treatment for the following periodontal conditions that may result in dental emergencies within the next 12 months.
 - 1) Edentulous areas or teeth requiring immediate prosthodontic treatment for adequate mastication or communication or acceptable esthetics.
 - 2) Active progressive moderate or advanced periodontitis.
 - 3) Periodontal abscess
 - 4) Progressive mucogingival condition
 - 5) Periodontal manifestations of systemic disease or hormonal disturbances
 - 6) Heavy subgingival calculus
- d. Edentulous areas or teeth requiring immediate prosthodontic treatment for adequate mastication or communication or acceptable esthetics.
- e. Unerupted, partially erupted or malposed teeth with historical, clinical or radiographic signs or symptoms of pathosis that are recommended for removal.
- f. Chronic oral infections or other pathologic lesions including:
 - 1) Pulpal, periapical or resorptive pathology requiring treatment.
 - 2) Lesions requiring biopsy or awaiting biopsy report.
- g. Emergency situations requiring therapy to relieve pain, treat trauma, treat acute oral infections or provide timely follow-up care (e.g., drain or suture removal) until resolved.

- h. Acute Temporomandibular disorders requiring active treatment that may interfere with duties.

Class 4: Patients who require periodic dental examinations or patients with unknown dental classifications. Class 4 patients normally are not considered to be worldwide deployable.

TECHNICAL EXHIBIT 5

Authorized / Unauthorized Dental Procedures

1. Dental Procedures authorized:

- a. Diagnostic Services funded through NG6H 2065 Medical Readiness Funding:
 - 1. D0120 – Periodic Oral Evaluation
 - 2. D0150 – Comprehensive Oral Evaluation
 - 3. D0330 – Panoramic Film
 - 4. D0272 – Bitewings – Two Films
 - 5. D0274 – Bitewings – Four Films
 - 6. D0220/D0230 – Periapical (limited)
- b. Restorative Services which are funded through the VFRE 2020 Dental Treatment funding (provided only to Soldiers alerted to an OCONUS Mission):
 - 1. D2140 – Amalgam – One Surface
 - 2. D2150 – Amalgam – Two Surfaces
 - 3. D2160 – Amalgam- Three Surfaces
 - 4. D2161 – Amalgam – Four or more surfaces
 - 5. D2330 – Composite – One Surface
 - 6. D2331 – Composite – Two Surfaces
 - 7. D2332 – Composite – Three Surfaces
- c. Posterior composites shall be authorized only when a posterior composite is necessary to provide a quality restoration and no other alternative is available.
 - 1. D2335 – Composite – Four or More Surfaces
 - 2. D2391 – Composite – One Surface

3. D2392 – Composite – Two Surfaces
 4. D2393 – Composite – Three Surfaces
 5. D2394 – Composite – Four or More Surfaces
 6. D2799 – Provisional Crown
 7. D2931 – Prefabricated Stainless-Steel Crown
 8. D2950 – Core Buildup, including any pins
 9. D2954 – Prefabricated Post and Core
- d. Restorative Services which are funded through the NG6H 2065 Funding (provided to Class 3 Soldiers not on alert status):
1. D2140 – Amalgam – One Surface
 2. D2150 – Amalgam – Two Surfaces
 3. D2160 – Amalgam – Three Surfaces
 4. D2161 – Amalgam – Four or More Surfaces
 5. D2330 – Composite- One Surface
 6. D2331 – Composite – Two Surfaces
 7. D2332 – Composite – Three Surfaces
- e. Endodontic Services which are funded through the VFRE 2020 Dental Treatment Funding (provided only to Soldiers alerted to an OCONUS Mission):
1. D3310 – Anterior Root Canal Therapy
 2. D3320 – Bicuspid Root Canal Therapy
 3. D3330 – Molar Root Canal Therapy
 4. D3346 – Anterior Root Canal Retreatment
 5. D3347 – Bicuspid Root Canal Retreatment
 6. D3348 – Molar Root Canal Retreatment
- f. Endodontic Services which are funded through the NG6H 2065 Funding (provided to Class 3 Soldiers not on alert status):
1. D3310 – Anterior Root Canal Therapy
 2. D3320 – Bicuspid Root Canal Therapy
 3. D3330 – Molar Root Canal Therapy
 4. D3346 – Anterior Root Canal Retreatment
 5. D3347 – Bicuspid Root Canal Retreatment
 6. D3348 – Molar Root Canal Retreatment
- g. Periodontal Services which are funded through the VFRE 2020 Dental Treatment Funding (provided only to Soldiers alerted to an OCONUS Mission):
1. D4355 – Full Mouth Debridement

2. D4342 – Periodontal Scaling and Root Planning, 1 to 3 teeth per quadrant

Periodontal scaling and root planning is authorized only when a periodontal abscess is present.

- h. Periodontal Services which are funded through the NG6H 2065 Funding (provided to Class 3 Soldiers not on alert status):
 1. D4355 – Full Mouth Debridement
 2. D4342 – Periodontal Scaling and Root Planning, 1 to 3 teeth per quadrant
- i. Prosthodontic Services which are funded through the VFRE 2020 Dental Treatment Funding (provided only to Soldiers alerted to an OCONUS Mission):
 1. D5110 – Complete Denture – Maxillary
 2. D5120 – Complete Denture – Mandibular
 3. D5211 – Maxillary Partial Denture –Resin Base
 4. D5212 – Mandibular Partial Denture – Resin Base
- j. Prosthodontic Services which are funded through the NG6H 2065 Funding (provided to Class 3 Soldiers not on alert status):
 1. D5110 – Complete Denture – Maxillary
 2. D5120 – Complete Denture – Mandibular
 3. D5211 – Maxillary Partial Denture –Resin Base
 4. D5212 – Mandibular Partial Denture – Resin Base
- k. Oral Surgery Services which are funded through the VFRE 2020 Dental Treatment Funding (provided only to Soldiers alerted to an OCONUS Mission):
 1. D7140- Extraction, Erupted Tooth or Exposed Root
 2. D7210 – Surgical Removal of Erupted Tooth
 3. D7220 – Removal of Impacted Tooth, Soft Tissue
 4. D7230 – Removal of Impacted Tooth, Partial Bony
 5. D7240 – Removal of Impacted Tooth, Completely Bony
- l. Oral Surgery Services which are funded through the NG6H 2065 Funding (provided to Class 3 Soldiers not on alert status):
 1. D7140- Extraction, Erupted Tooth or Exposed Root
 2. D7210 – Surgical Removal of Erupted Tooth
 3. D7220 – Removal of Impacted Tooth, Soft Tissue
 4. D7230 – Removal of Impacted Tooth, Partial Bony

5. D7240 – Removal of Impacted Tooth, Completely Bony

2. Orthodontic Retention

a. Unauthorized Dental Procedures

1. Bridges/and or implants
2. Removal of Asymptomatic third molars
3. Routine Prophylaxes (cleanings)
4. Restorations for esthetics only
5. Restorations for small (non-class 3) caries
6. Extensive Root plane/scaling
7. Osseous Surgery
8. Other diagnostic services

- b. The above listings are guides to assist the State Dental Officer or his/her designee in determining treatment to be authorized. The treatment provided by the ARNG is to get the Soldier from a dental class 3 to a dental class 2 only. Comprehensive Treatment (to dental class 1) is not authorized.
- c. If a Soldier needs all of his/her remaining teeth in an arch extracted and a denture placed, the Soldier shall not be available for deployment for at least 6 months and shall be disqualified from immediate deployment. However, if the Soldier is otherwise deployable (passes all medical, personnel and training requirements) and, after extraction and placement of prosthesis, would be deployable, the dental treatment could be provided, and the Soldier could be considered as a replacement at a later date.
- d. If a Soldier shall have a number of teeth extracted to the extent mastication and speech are adversely affected, then an acrylic removable partial denture shall be authorized if there is sufficient time for healing and construction of the partial denture prior to departure to Mobilization Stations.
- e. Soldiers requiring extensive periodontal treatment shall be disqualified from deployment and educated on the TRICARE Dental Program. If he/she obtains the required therapy (from his/her private dental provider) and reaches a maintenance level, then he/she would become deployable.